



Laboratory Investigation Report

Patient Name	: Kiran Tiwari	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 47 Y 3 M 0 D /F	OP/IP No	: OP/96450
Max ID/Mobile	: 214858/8752005200	Collection Date/Time	: 10/Aug/2021 07:24PM
Lab ID	: 1266082100239	Receiving Date	: 10/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 10/Aug/2021

Clinical Biochemistry

Creatinine, Serum*

Date	10/Aug/2021 07:24PM	Unit	Bio Ref Interval
Creatinine	0.75	mg/dl	0.61 - 1.24
Rate-Jaffe			
eGFR	82.74	ml/min/1.73 m ²	
MDRD			

Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs ≤ 60 ml / min / 1.73 m². MDRD equation is **used for adult population only.**

<60 ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min / 1.73 m² - Kidney failure

Blood Sugar (Random), Fluoride Plasma*

Date	10/Aug/2021 07:24PM	Unit	Bio Ref Interval
Random Glucose	93.11	mg/dl	74 - 140
UV-Hexokinase			

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



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SIN No: B2B845123, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752

The : Max Super Specialty Hospital, Saket (West Block)
1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

CIN No.: L72200MH2001PLC322854

www.maxlab.co.in

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Clinical Biochemistry

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Clinical Pathology

Urine Routine And Microscopy*

Date	10/Aug/2021 07:24PM	20/Oct/20 01:07PM	30/Sep/20 08:21AM	Unit	Bio Ref Interval
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Macroscopy

Reflectance photometry

Colour	Pale Yellow	Pale Yellow	Pale Yellow		Pale Yellow
PH	5.0	5.5	6.5	..	5-6
Specific Gravity	1.020	1.010	1.015		1.015 - 1.025
Protein	Nil	Nil	Nil		Nil
Glucose.	Nil	Nil	Nil		Nil
Ketones	Nil	Nil	Nil		Nil
Blood	Nil	Nil	Nil		Nil
Bilirubin	Nil	Nil	Nil		Nil
Urobilinogen	Normal	Normal	Normal		Normal
Nitrite	Negative	Negative	Negative		

Microscopy

Light Microscopy/Image capture microscopy

Red Blood Cells (RBC)	Nil	Nil	Nil	/HPF	Nil
White Blood Cells	1-2	2-3	1-2	/HPF	0.0-5.0
Squamous Epithelial Cells	1-2	5-7	1-2	/HPF	
Cast	Nil	Nil	Nil	/LPF	Nil
Crystals	Nil	Nil	Nil	..	Nil
Bacteria	Nil	Nil	Nil	/HPF	Nil

Kindly correlate with clinical findings

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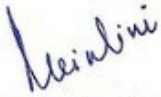
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Clinical Pathology

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Hematology

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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	10/Aug/2021 07:28PM	20/Oct/20 01:07PM	30/Sep/20 08:21AM	Unit	Bio Ref Interval
Haemoglobin Modified cyanmethemoglobin	13.16	12.67	12.88	g/dl	12.0 - 15.0
Packed Cell, Volume Calculated	39.9	37.1	37.9	%	40-50
Total Leucocyte Count (TLC) Electrical Impedance	26.76	12.40	13.41	10~9/L	4.0-10.0
RBC Count Electrical Impedance	3.90	3.92	4.02	10~12/L	3.8-4.8
MCV Electrical Impedance	102.4	94.7	94.3	fL	83-101
MCH Calculated	33.7	32.3	32.0	pg	27-32
MCHC Calculated	33.0	34.2	34.0	g/dl	31.5-34.5
Platelet Count Electrical Impedance	501.1	478.0	503	10~9/L	150-410
MPV Calculated	10.00	10.24	10.19	fL	7.8-11.2
RDW Calculated	12.8	13.2	13.7	%	11.5-14.5

Differential Cell Count

VCS / Light Microscopy

Neutrophils	80	73	72	%	40-80
Lymphocytes	14	20	18	%	20-40
Monocytes	06	06	08	%	2-10
Eosinophils	00	01	02	%	1-6
Basophils	00	00	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	21.41	9.05	9.66	10~9/L	2.0-7.0
Absolute Lymphocyte Count	3.8	2.5	2.4	10~9/L	1.0-3.0
Absolute Monocyte Count	1.61	0.74	1.07	10~9/L	0.2-1.0



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Hematology

Kindly correlate with clinical findings

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Immunoassay

Total - Thyroid Profile*, Serum

Date	10/Aug/2021 05:24PM	Unit	Bio Ref Interval
T3 (Total) CLIA	1.12	ng/mL	0.87-1.78
T4 (Total) CLIA	7.33	µg/dL	6.09-12.23
TSH Chemiluminescence	3.09	uIU/ml	0.34-5.6

Comment

Parameter	Unit	Cord Blood	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	2.3 - 13.2	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.

Decreased in primary Hyperthyroidism

Total Thyroid Profile : (Thyroid Function Test, TFT)

T3 (Total), Triiodothyronine

Increase in Hyperthyroidism, and T3 toxicosis,

Decreased in hypothyroidism, states with decreased TBG, and acute and subacute non thyroidal illness

T4(Total) Thyroxine

Increased in Hyperthyroidism, states with increased TBG, Thyrotoxicosis

Decreased in Hyperthyroidism, states with decreased TBG and Strenuous exercise

TSH, Serum : Thyrotropin(Thyroid Stimulating Hormone)

Increased in primary Hypothyroidism.

Decreased in primary Hyperthyroidism.

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am

and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence

time of the day has influence on the measured serum TSH concentrations.

TSH assay is standardized to the 3rd generation for human TSH.

The Cyclical variations may be quite large; therefore the timing of specimen collection must be strictly controlled.

Advise : Kindly do Thyroid Profile/TSH in morning hours only.



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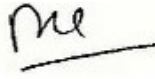
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Immunoassay

Kindly correlate with clinical findings

***** End Of Report *****

Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology





Laboratory Investigation Report

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Microbiology

Urine Culture & Sensitivity

Method : Semi quantitative-Culture/ID & Sensitivity by Vitek 2

Result No growth.
Interpretation. Sterile after Overnight/24 hours of aerobic incubation at 37 degree C.

Comment

Urine pus cells /HPF	Colony count	Interpretation
<5	10 ³	Insignificant growth, more likely to be a colonizer. To be treated only if the patient is symptomatic
<5	10 ⁴	Moderately significant growth. Should be correlated clinically and to be treated only if the patient is symptomatic
<5	10 ⁵	Significant growth. Should be treated if the patient is clinically symptomatic
>5	10 ³ / 10 ⁴ / 10 ⁵	Significant growth. Should be treated if the patient is clinically symptomatic
5 - 10	No growth	Kindly rule out the cause of sterile pyuria i.e Is the patient on antibiotics Or anyother systemic illness (e.g TB /STD)
>5	Mixed growth	Mixed growth of more than two types of organisms indicating specimen colonization. Kindly send mid-stream urine sample after proper collection.

Kindly correlate with clinical findings

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Microbiology



Dr. Bansidhar Tarai, M.D.

Associate Director

Microbiology & Molecular Diagnostics



Dr. Poornima Sen, M.D.

Consultant - Microbiology



Dr. Madhuri Somani, M.D. , DNB

Consultant - Microbiology



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